Flow story

Human in the loop: the nurse enters data at the start, the clinician approves at the gate.

1. The case is ingested and the fields are validated.

2. The local offline model turns the complaint into typed fields.

3. The triage server matches SATS discriminators for the complaint.

4. Deterministic code computes TEWS and returns a colour with citations.

5. A verifier rejects any priority that arrives without a citation.

6. Worsening vitals raise a second look flag, never a reclassification.

7. The case pauses until a named clinician approves, edits, or dismisses it.

8. Only after approval is a queue position written, by the gated tool.

9. A sourced case card is rendered for the nurse.

Every tool call is written to the hash chained audit log. The whole path runs offline on synthetic data, with no external API.

TriageDesk: Architecture and Flow Story

Data sources

synthetic fixtures

Human in the loop

Triage nurse
enters vitals and a one line complaint

LangGraph Agent (orchestrator)

1. ingest
validate the case

2. normalize
structure the complaint

3. retrieve
match SATS discriminators

4. score
deterministic TEWS + citations

5. verify
reject any uncited priority

6. recheck
flag a second look, never reclassify

7. approval gate
pause for a named clinician

8. commit
gated queue write

9. handover
render the sourced case card

structured output

critic

checkpoints.db
runtime state

MCP client

langchain.mcp adapter
stdio

no network

External APIs: none (offline)

Owned Triage MCP server

Prompts

normalize_case

verify_priority

Resources (read-only)

sats://tews

sats://discriminators

sats://discriminators/{id}

sats://version

Tools (act)

normalize_case

commit_priority (gated)

match_discriminators

score_priority

raise_recheck_flag

Borrowed SQLite MCP server

query tools

table resources

data/sats_tews.yaml

data/sats_discriminators.yaml

log/audit.jsonl
hash chain

triage.db
cases + queue + audit

Local LLM (offline)

Ollama: Qwen3 4B
fallback Qwen2.5
structured JSON output

Named clinician
approves, edits, or dismisses